SAMPLE CITY GENERAL HOSPITAL (FICTIONAL)

Department of Pulmonary Medicine & Respiratory Diseases | Designated Microscopy Centre (DMC)
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MEDICAL REPORT: PULMONARY TUBERCULOSIS

1. PATIENT DETAILS

Patient Name	Mr. Rahul Verma (fictional)	**UHID / Reg. No.**	SCGH-2026-004512
Age / Sex	34 Years / Male	**Date of Report**	31-Oct-2026
Weight / Height	58 kg / 170 cm (BMI 20.1)	**Ni-kshay ID**	NIK-0000-0000 (sample)
Address	Sample Colony, Sample City	**Contact**	+91 00000 00000
Referred By	Dr. A. Sample, General Medicine OPD	**Treating Doctor**	Dr. S. Example, MD (Pulm. Med.)

2. PRESENTING COMPLAINTS & HISTORY

- Cough with expectoration for **4 weeks**; occasional blood-streaked sputum for 5 days.
- Evening rise of temperature (low-grade fever) with night sweats for 3 weeks.
- Loss of appetite and weight loss of about 5 kg in 2 months; generalised weakness.
- Mild right-sided chest pain on deep breathing. No breathlessness at rest.
- **Contact history:** Colleague at workplace treated for TB 8 months ago.
- **Past history:** No previous TB treatment. No diabetes, hypertension or asthma. No known drug allergy.
- **Personal history:** Non-smoker, no alcohol. HIV status: tested NEGATIVE (see investigations).

3. CLINICAL EXAMINATION

Parameter	Finding	Parameter	Finding
Temperature	99.8 F	Pulse	92 / min, regular
Blood Pressure	118 / 76 mmHg	Respiratory Rate	20 / min
SpO2 (room air)	96 %	General Condition	Thin built, mild pallor, no clubbing, no lymphadenopathy
Respiratory System	Reduced air entry and bronchial breath sounds in right upper zone; coarse crepitations present	Other Systems	CVS, CNS, abdomen: within normal limits

4. INVESTIGATIONS

Test	Result	Reference / Remark
Sputum AFB smear (2 samples)	POSITIVE (2+ / 1+)	Negative
CBNAAT / Xpert MTB-RIF	MTB DETECTED; Rifampicin resistance NOT detected	Drug-sensitive TB
Chest X-ray (PA view)	Patchy infiltrates with a small cavity in right upper zone; no pleural effusion	Abnormal
Mantoux (TST) 48-72 h	16 mm induration	>= 10 mm positive
Hemoglobin	11.2 g/dL	13.0 - 17.0 g/dL (mildly low)
Total WBC / Lymphocytes	9,800 /cu mm / 34 %	4,000 - 11,000 /cu mm
ESR (1st hour)	58 mm/hr	0 - 15 mm/hr (raised)
Random Blood Sugar	96 mg/dL	< 140 mg/dL
HIV 1 & 2 (ELISA)	Non-reactive	Non-reactive
LFT (baseline)	SGOT 28 U/L, SGPT 31 U/L, Bilirubin 0.7 mg/dL	Within normal limits
Serum Creatinine	0.9 mg/dL	0.7 - 1.3 mg/dL

5. DIAGNOSIS

Final Diagnosis	**Pulmonary Tuberculosis, bacteriologically confirmed, NEW case, drug-sensitive (Rifampicin sensitive)**

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ICD-10 Code	A15.0 (TB of lung, confirmed by sputum microscopy)
Category	New case (no prior ATT). Case notified on Ni-kshay portal as per national guidelines.

6. TREATMENT PLAN (Standard Daily Regimen: 2HRZE + 4HR, total 6 months)

Phase	Medicine (Fixed-Dose Combination)	Dose (weight band 55-69 kg)	Duration	Timing
Intensive	Isoniazid 75 mg + Rifampicin 150 mg + Pyrazinamide 400 mg + Ethambutol 275 mg (HRZE)	4 tablets once daily	2 months (56 days)	Early morning, empty stomach with water
Continuation	Isoniazid 75 mg + Rifampicin 150 mg (HR)	4 tablets once daily	4 months (112 days)	Early morning, empty stomach with water
Supportive	Pyridoxine (Vitamin B6)	10 mg once daily	Throughout treatment	After breakfast (prevents nerve side effects)
Supportive	Multivitamin + Iron/Folic acid tablet	1 tablet once daily	1 month, then review	After lunch
Symptomatic	Cough suppressant syrup (as needed)	10 ml up to 3 times/day	5 - 7 days	Only if dry cough troubles sleep

Doses follow weight-band dosing under the national TB programme (NTEP). Medicines are given free of cost; treatment is observed by a DOT provider / health worker. Never stop or change medicines without consulting the doctor.

7. EXPECTED SIDE EFFECTS & WHEN TO CONTACT DOCTOR

- **Common and harmless:** orange/red colouration of urine, sweat and tears (due to Rifampicin); mild nausea; joint ache.
- **Contact the hospital the same day if:** yellowing of eyes or skin, persistent vomiting, severe abdominal pain, dark urine (liver problem).
- Numbness/tingling in hands or feet; blurring of vision or difficulty seeing colours (Ethambutol); skin rash or itching; ringing in ears.
- Coughing up more blood, high fever, severe breathlessness or chest pain: **go to the emergency department immediately.**

8. PRECAUTIONS & ADVICE

Prevent spread to others

- Cover mouth and nose with a cloth or elbow while coughing or sneezing.
- Wear a mask in public and around family members for first 2-3 weeks of treatment.
- Never spit in open; use a closed container with disinfectant or tissue and dispose safely.
- Keep room well-ventilated; sunlight and fresh air kill TB bacteria.
- Sleep in a separate, airy room if possible during the intensive phase.
- Wash hands regularly; do not share utensils, towels or bedding.
- Family members and close contacts should be screened (symptoms, chest X-ray). Children under 6 years in contact may need preventive therapy.

Care of yourself

- Take medicines **every day without a gap**, even when you feel better. Missed doses can cause drug-resistant TB.
- Avoid alcohol completely; it damages the liver together with TB drugs.
- Do not smoke or use tobacco in any form.
- Do not take other medicines, herbal or ayurvedic products without asking your doctor.
- Take adequate rest for the first 2 months; resume light activity gradually.
- Complete the full 6-month course even if symptoms disappear in 2-3 weeks.
- Keep the treatment card safe and carry it on every visit.

9. DIET & NUTRITION

- High-protein, calorie-rich diet: milk, curd, eggs, pulses, lentils, paneer, soya, nuts, lean meat or fish.
- Plenty of fruits and green vegetables (vitamin C, iron, folate); drink 2-3 litres of water daily.
- Eat small frequent meals if appetite is poor. Nutritional support (Ni-kshay Poshan Yojana) may be available for eligible patients.
- Avoid junk food, excess oil and spice; avoid alcohol and tobacco.

10. FOLLOW-UP SCHEDULE

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Visit	When	What will be done
Visit 1	End of Month 1	Clinical review, weight, adherence check, LFT if symptomatic
Visit 2	End of Month 2 (Intensive phase)	Sputum smear repeat; switch to continuation phase if negative
Visit 3	End of Month 4	Clinical review, sputum smear, weight monitoring
Visit 4	End of Month 6 (End of treatment)	Sputum smear, chest X-ray, declare outcome (Cured / Treatment completed)
Post-treatment	6 and 12 months after completion	Clinical check-up to detect any relapse

11. PROGNOSIS & REMARKS

Prognosis is **good** with regular and complete treatment. Patient is advised rest, nutritious diet and no work involving close public contact for 2-3 weeks. Patient counselled about the disease, its spread, drug side effects, importance of adherence and need for follow-up. Patient and family verbalised understanding.

Dr. S. Example, MD (Pulmonary Medicine)
Reg. No. XXXXX (fictional)
Consultant, Dept. of Pulmonary Medicine
(Sample signature & seal)

Date: 31-Oct-2026
Place: Sample City

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